NCCP 00376 · VERSION 4

# Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

## Make treatment fit into daily life

Swallow the tablet whole, preferably after a meal; do not chew or crush it. Skip a missed dose and continue at the next scheduled time; do not double the dose.

## Take after food

A regular meal can anchor the daily dose. Do not compensate for a missed tablet with a double dose.

## Count the whole endocrine course

In an adjuvant switch, the earlier tamoxifen years count towards the intended total. Switching medicines does not automatically restart a five-year clock.

## Bone and vitamin D

Ask about baseline DXA, vitamin D assessment if appropriate, and the result-review plan. Supplements or antiresorptive drugs should follow the individual plan.

## Medicine interactions

Strong enzyme-inducing medicines can reduce exposure. Ask the pharmacy to check all medicines and supplements before starting or changing them.

## Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

## Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.


- NCCP 00376 v4 · official regimen
- HPRA Exemestane SmPC cited by NCCP
- Long-term follow-up of the Intergroup Exemestane Study
- Pan-Asian adapted ESMO early breast cancer guidelines
- Pan-Asian adapted ESMO metastatic breast cancer guidelines
- NICE NG 101: early and locally advanced breast cancer
- ESO-ESMO ABC 5 consensus: tumour-marker interpretation
- IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss
- eviQ 3650 v 6: adjuvant whole-breast radiotherapy
- Exemestane versus megestrol after tamoxifen failure in advanced breast cancer


Source reconciled; independent clinical, pharmacy and patient/PPI review pending